

R3 Recovery Services

618 Columbia Ave, Lancaster, Pennsylvania, US, 17603
(717) 349-9924

Date: 06/17/2026

CLIENT PROFILE

Primary Clinician: Marleigh Johnson
Status: Active
Admission Date: 05/01/2026
Discharge Date: 05/01/2026
Medical Provider:
Level Of Care: IOP 5
Treatment:

Code Status: NA

Gender: Female
Pronoun(s): She/her/hers
Birth Date: 12/08/2001

MEDICATIONS

Medication	Medication Type	Dosage	Instructions	Status	Start Date	End Date
Apri	Home	0.15-0.03 mg tablet by mouth	Take 0.03-0.15 mg by mouth	Active	05/14/2026	
buspirone	Home	10 mg tablet by mouth	2x per day 20 mg total per day Take 10 mg by mouth	Active	05/14/2026	
levothyroxine	Home	75 mcg tablet by mouth	Take 75 mcg by mouth	Active	05/14/2026	
Lexapro	Home	20 mg tablet by mouth	Take 20 mg by mouth	Active	05/14/2026	
semaglutide	Home	0.25 mg or 0.5 mg (2 mg/3 mL) Pen Injector subcutaneous	Take 0.1 Unit(s) subcutaneous	Active	05/14/2026	
trazodone (bulk)	Home	powder miscellaneous	Take 50 Capsule(s) miscellaneous 2x per day	Active	05/14/2026	

PRN MEDICATIONS

Medication	Medication Type	Dosage	Instructions	Status	Start Date	End Date
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ALLERGIES

-- No Allergies --

Fetching Report ...

DIAGNOSIS

Diagnostic Impression	Priority	Created By	Created Date	Resolved
[F14.20] Cocaine use disorder, Severe	Primary	Caleb Madison	05/01/2026	--
[F10.20] Alcohol use disorder, Severe	Secondary	Caleb Madison	05/01/2026	--

VITALS

-- No Vitals --

INSURANCE

Insurance	Subscriber	Relationship	Plan	Policy No.	Priority	Co-Pay	Status
United Health Care		Self	None Specified	51695708102	Primary		Active

AUTHORIZATION

-- No Authorizations --

PRIVATE PAY

-- No Private Pay --

SUBSTANCE USE HISTORY

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Cocaine	0	snorting	2 grams	03/07/2026	22	2	getting it from a dealer	Continuous
Alcohol	0	Drinking	Bottle of wine	03/06/2026	16	8	buy it	Continuous

TESTING ANALYSIS

-- No Testing Analysis --

CLINICAL OPIATE WITHDRAWAL SCALE

-- No COWS --

CLINICAL INSTITUTE WITHDRAWAL ASSESSMENT

-- No CIWAs --

CLINICAL INSTITUTE WITHDRAWAL ASSESSMENT - BENZODIAZEPINES

-- No CIWA-Bs --

Fetching Report ...

SHIFT NOTES

-- No Notes --

SHIFT ROUNDS

-- No Shift Rounds --

DOCTORS ORDERS

-- No Doctors Orders --

PROGRESS NOTES

-- No Progress Notes --

Fetching Report ...

INDIVIDUAL APPOINTMENTS

06/12/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Attended	8:00AM	9:00AM
Facility							
R3 Recovery Services							

Treatment Objectives:

[REDACTED] will explore personal relationship patterns and emotional needs in group and individual sessions.

Assessment Note:

Data:
Client reflected on ongoing interpersonal challenges, specifically related to her relationships with men. Client identified a pattern of prioritizing these relationships over her own needs and acknowledged difficulty in valuing herself independently of male attention. Client processed how this dynamic presents in her life and the emotional impact it has on her self-worth. Client disclosed insight into her substance use risk, stating that if prescribed Adderall, she would likely misuse it by snorting it in order to "get high without getting caught on a drug test." Client identified awareness of this thought pattern and expressed a desire to change it, noting that sharing it in session felt important for accountability. This disclosure was processed in session with a focus on insight and risk awareness. Client additionally discussed feeling that while she has a clear understanding of her preferences and likes, she struggles with a sense of identity and does not feel she knows who she is as a person.

Assessment:
Client demonstrates growing insight into maladaptive relational patterns and substance use thought processes, including awareness of potential medication misuse and the underlying intent to circumvent accountability. This level of honesty indicates increasing engagement in treatment and willingness to [REDACTED] ept, particularly in the absence of validation from relationships with men. These patterns contribute to decreased self-esteem [REDACTED] appropriate for current level of care, as ongoing therapeutic support is needed to [REDACTED] al patterns related to both relationships and substance use.

Plan:
Continue to support client in exploring identity development separate from relationships, with focus on strengthening self-worth and internal validation. Therapist will utilize cognitive restructuring and motivational interviewing to [REDACTED] nces. Continue exploration of relationship patterns and development of healthier boundaries and self-prioritization. C [REDACTED] ntention to deepen self-exploration and identity formation. Follow up in next session to process artwork and assess progress in shifting thought patterns and behaviors.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Into
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☒ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

☐ WNL(within normal limit)	☐ Clean	☐ Unshaved	☐ Odorous	☐ Disheveled	☐ Other
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Affect

☐ WNL(within normal limit)	☐ Appropriate	☐ Inappropriate	☐ Labile	☐ Blunted	☐ Flat
☐ Other	☐ Restricted	☐ Expansive	☐ Euphoric	☐ Anxious	☐ Angry

Hallucinations

☐ WNL(within normal limit)	☐ Visual	☐ Auditory	☐ Tactile	☐ Olfactory	☐ Command
	☐ Denied	☐ Other			

Movement

☐ WNL(within normal limit)	☐ Normal	☐ Tense	☐ Fidgety	☐ Hyperactive	☐ Psychomotor
☐ Psychomotor Agitation	☐ Pacing	☐ Hand Wringing	☐ Tics	☐ Other	Retardation

Fetching Report ...

Mental Status Examination

Mood

WNL(within normal limit) Normal	Anxious Euphoric Other	Confused Angry	Depressed Dysthymic	Labile Irritated	Fearful Euthymic
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Speech

WNL(within normal limit)	Normal Slowed	Soft Slurred	Loud Incoherent	Pressured	Rapid
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Impulse Control

WNL(within normal limit)	Good	Limited	Poor
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Insight

WNL(within normal limit)	Good	Limited	Poor
--	----------------------------	-------------------------------	----------------------------

Intelligence

WNL(within normal limit)	Above Average	Average	Below Average
--	-------------------------------------	-------------------------------	-------------------------------------

Oriented

WNL(within normal limit)	Person	Place	Time	Situation
--	------------------------------	-----------------------------	----------------------------	---------------------------------

Other

WNL(within normal limit)	Obsessions	Compulsions	Depersonalization	DeRealism	Denied
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Memory

WNL(within normal limit)	Intact	Immediate Impaired	Short Term Impaired	Long Term Impaired
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Suicidal Ideation

WNL(within normal limit)	Yes	Plan	Intent	Means	Denied
--	---------------------------	----------------------------	------------------------------	-----------------------------	------------------------------

Homicidal Ideation

WNL(within normal limit)	Yes	Plan	Intent	Means	Denied
--	---------------------------	----------------------------	------------------------------	-----------------------------	------------------------------

Eye Contact

WNL(within normal limit)	Good	Avoidant	Fleeting	Stare
--	----------------------------	--------------------------------	--------------------------------	-----------------------------

Facial Expression

WNL(within normal limit)	Amiable Sad	Happy	Angry	Worried	Suspicious
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Thought Process

Fetching Report ...

Mental Status Examination

WNL(within normal limit)	Intact Flight	Blocked Circumstantial	Loose Other	Incoherent	Tangential
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Delusions

WNL(within normal limit)	Grandiose Denied	Persecutory Other	Somatic	Paranoia	Ideas
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Demeanor

WNL(within normal limit)	Cooperative Friendly Agitated Other	Uncooperative Worried Evasive	Guarded Boastful Dramatic	Resistive Demanding Seductive	Defensive Hostile Arrogant
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Attention/Concentration

WNL(within normal limit)	Satisfactory	Poor	Easily Distracted	Other
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Judgement

WNL(within normal limit)	Good	Limited	Poor
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Staff

Marleigh Johnson, MS, LPC
06/12/2026 1:15PM

06/16/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Individual Session	Individual Session	Marleigh Johnson, MS, LPC	--	60 Min	Attended	8:00AM	9:00AM
Facility R3 Recovery Services							

Treatment Objectives:

[REDACTED] will identify emotional triggers and physiological signs of distress during group sessions.

Assessment Note:

Data:
Client attended scheduled individual session and was engaged and cooperative throughout. Client reported ongoing struggles with managing finances, acknowledging a pattern of avoidance and ignoring financial responsibilities, which has contributed to increased stress. Client verbalized increased readiness to [REDACTED] for individual, which may provide a more financially sustainable living arrangement. Client expressed motivation to attend th [REDACTED] processed a past friendship, identifying ongoing emotional impact and recognizing the need to end the relationship due to its negative influence on her well-being. Client demonstrated insight into the emotional weight of this decision. Client reported she has not yet completed her assigned homework of painting an image reflecting who she wants to become but acknowledged its importance.

Assessment:
Client is demonstrating increased insight into maladaptive avoidance behaviors related to financial stress and is progressing toward readiness for behavioral change. Her willingness to explore alternative housing options and engage in financial education indicates movement into a more action-oriented stage of change. Client continues to process interpersonal boundaries, specifically related to unhealthy relationships, and is beginning to prioritize emotional safety and stability. However, ongoing avoidance of assigned therapeutic tasks (painting homework) indicates some resistance and difficulty with follow-through. Overall, client presents as motivated but continues to require support in translating insight into consistent action.

Plan:
Therapist will continue to support client in developing practical financial management skills and reinforce participation in finance group to build accountability and structure around budgeting behaviors. Future sessions will focus on identifying and [REDACTED] on responsibilities. Therapist will also continue to process the emotional impact of interpersonal

Fetching Report

relationships and support client in establishing and maintaining healthy boundaries. Client will complete the assigned painting homework prior to next session to further explore identity development and future goals. Client will continue attending scheduled sessions and groups as recommended.

Select the dimension of ASAM

- ☐ Dimension 1: Acute Info
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☐ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Mental Status Examination

Hygiene

WNL(within normal limit)

Clean

Unshaved

Odorous

Disheveled

Other

Affect

WNL(within normal limit)

Appropriate

Inappropriate

Labile

Blunted

Flat

Restricted

Expansive

Euphoric

Anxious

Angry

Other

Hallucinations

WNL(within normal limit)

Visual

Auditory

Tactile

Olfactory

Command

Denied

Other

Movement

WNL(within normal limit)

Normal

Tense

Fidgety

Hyperactive

Psychomotor Retardation

Psychomotor Agitation

Pacing

Hand Wringing

Tics

Other

Mood

WNL(within normal limit)

Anxious

Confused

Depressed

Labile

Fearful

Euphoric

Angry

Dysthymic

Irritated

Euthymic

Normal

Speech

WNL(within normal limit)

Normal

Soft

Loud

Pressured

Rapid

Slowed

Slurred

Incoherent

Impulse Control

WNL(within normal limit)

Good

Limited

Poor

Insight

WNL(within normal limit)

Good

Limited

Poor

Intelligence

WNL(within normal limit)

Above Average

Average

Below Average

Oriented

Fetching Report ...

Mental Status Examination

WNL(within normal limit)	Person	Place	Time	Situation	
Other					
WNL(within normal limit)	Obsessions	Compulsions	Depersonalization	DeRealism	Denied
Memory					
WNL(within normal limit)	Intact	Immediate Impaired	Short Term Impaired	Long Term Impaired	
Suicidal Ideation					
WNL(within normal limit)	Yes	Plan	Intent	Means	Denied
Homicidal Ideation					
WNL(within normal limit)	Yes	Plan	Intent	Means	Denied
Eye Contact					
WNL(within normal limit)	Good	Avoidant	Fleeting	Stare	
Facial Expression					
WNL(within normal limit)	Amiable Sad	Happy	Angry	Worried	Suspicious
Thought Process					
WNL(within normal limit)	Intact Flight	Blocked Circumstantial	Loose Other	Incoherent	Tangential
Delusions					
WNL(within normal limit)	Grandiose Denied	Persecutory Other	Somatic	Paranoia	Ideas
Demeanor					
WNL(within normal limit)	Cooperative Friendly Agitated Other	Uncooperative Worried Evasive	Guarded Boastful Dramatic	Resistive Demanding Seductive	Defensive Hostile Arrogant
Irritable					
Apathetic					
Attention/Concentration					
WNL(within normal limit)	Satisfactory	Poor	Easily Distracted	Other	
Judgement					
WNL(within normal limit)	Good	Limited	Poor		

Staff

[Redacted Staff Name]

Fetching Report ...



Marleigh Johnson, MS, LPC
06/16/2026 7:55AM

06/16/2026

Type	Title	Appt. With	Add-On Service	Duration	Status	Start Time	End Time
Case Management	Vocational Support	Kassandra Maharjan	--	30 Min	Incomplete	1:00PM	1:30PM
Facility							
R3 Recovery Services							

Assessment Note:

Fetching Report ...

GROUP APPOINTMENTS

06/10/2026

Appt. With	Title	Topic	Date
Saffron Brocious, BA	Wednesday Women's Group	Group Therapy	06/10/2026

Add-On Service
--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	10	9:00 AM - 12:00 PM

Treatment Objectives
-- No Treatment Objectives is provided --

Group Notes
Clients were given the opportunity to check in and share any updates or challenges they want to discuss with the group. They were prompted to share their energy levels on a scale of 1-10. Clients participated in an activity where facilitator read various statements related to the pressures women face as mothers, wives, women in recovery, women in the workforce, etc. Clients stood in a circle and stepped in if they agreed with each statement. They processed the activity and then discussed in more detail the specific issues they face in each domain of being a woman.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note
-- No assessment note is provided. --

Staff	Reviewer
Saffron Brocious, BA 06/15/2026 1:37PM	Marleigh Johnson, MS, LPC 06/15/2026 1:44PM

06/10/2026

Appt. With	Title	Topic	Date
Caleb Madison, BS	Spirituality Workshop	Group Therapy	06/10/2026

Add-On Service
--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	9	3:00 PM - 6:00 PM

Treatment Objectives
[REDACTED] will develop coping strategies for managing stress and emotional pain in group and individual sessions.
[REDACTED] will identify emotional triggers and physiological signs of distress during group sessions.

Group Notes
Data:
Group session focused on processing the theme of emotional stagnation and resistance to change using the song “Happy” by NF as a discussion prompt. Members were attentive and engaged as lyrics were reviewed, highlighting internal conflict between a desire for relief and a fear of losing one's identity if emotional patterns shift. Several members identified with the concept of becoming “comfortable in misery,” acknowledging a tendency to remain in familiar patterns of depression, anxiety, or substance use despite negative consequences. Clients verbalized difficulty asking for help, citing pride, shame, and fear of judgment as barriers.
Group members processed unresolved personal issues referenced in the song, including strained family relationships, avoidance behaviors, and un [REDACTED] difficult conversations and unresolved guilt or regret as contributors to ongoing emotional distress. There was observable insight among the group [REDACTED] maladaptive coping strategies.

Fetching Report ...

Discussion also explored the fear of change and uncertainty about identity in recovery, with members expressing concern about “not knowing who they would be” without their current struggles. Several members were able to connect this fear to relapse risk and resistance to fully engaging in recovery-oriented behaviors. Clients demonstrated the ability to reflect on their own emotional patterns and began identifying specific areas where they may be avoiding growth or healing. Group participation was active, with members providing peer support, validation, and shared experiences.

Patient Exercise

-- No patient exercise note is provided --

Assessment Note

Assessment:
Member demonstrates insight into diminished ability to experience and attend to positive stimuli, consistent with cognitive patterns of selective attention toward stressors and negative details. She verbalizes awareness that she becomes preoccupied with minor challenges, which interferes with her ability to appreciate positive aspects of her environment. This reflects elements of cognitive filtering and possible anhedonia or reduced emotional responsiveness to previously enjoyed experiences. Member’s expressed desire to “see more beauty in life” indicates motivation for change and increased openness to shifting perspective. Insight is present, though consistent application of adaptive cognitive strategies remains limited.

Plan:
Continue to utilize cognitive-behavioral interventions to [REDACTED] mindfulness-based strategies aimed at increasing present-moment awareness and appreciation of positive experiences. Encourage [REDACTED] emotional shift toward adaptive perspectives. Support member in identifying specific triggers that lead to fixation on minor stressors and develop strategies to reframe or redirect attention. Reinforce behavioral activation techniques that promote engagement in meaningful and enjoyable activities. Monitor progress in member’s ability to recognize and appreciate positive aspects of daily life.

Select the dimension of ASAM [REDACTED]

- ☐ Dimension 1: Acute Into
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff

Caleb Madison, BS
06/10/2026 5:33PM

Reviewer

Marleigh Johnson, MS, LPC
06/15/2026 1:45PM

06/11/2026

Appt. With	Title	Topic	Date
Marleigh Johnson, MS, LPC	Brain Chemistry Workshop	Group Therapy	06/11/2026

Add-On Service

--

Duration	Status	Group Attendees	Session Times
180 Min	Attended	10	9:00 AM - 12:00 PM

Treatment Objectives

[REDACTED] will practice mindfulness and grounding techniques in individual therapy to manage emotional intensity.

Group Notes

Data:
Group session focused on psychoeducation regarding the vagus nerve and its role in emotional regulation, stress response, and overall nervous system functioning. Clients were provided with information on how the vagus nerve impacts the body’s ability to move between states of activation (fight/flight) and relaxation (rest/digest). Discussion included how trauma, anxiety, and substance use can dysregulate the nervous system and contribute to difficulty managing emotions. Clients engaged in discussion about personal experiences with stress responses and identified physical and emotional cues associated with dysregulation. Facilitator introduced basic grounding and vagal regulation strategies (e.g., deep breathing, mindfulness, progressive relaxation) and encouraged group members to reflect on how these techniques could be incorporated into daily routines to support recovery and emotional stability.

Patient Exercise

Fetching Report ...

-- No patient exercise note is provided --

Assessment Note

Assessment:
Client demonstrates emerging insight into the connection between her childhood experiences and her current tendency to operate in a fight-or-flight state. Client reflected on early life stressors and how these experiences have shaped her emotional responses, suggesting a longstanding pattern of heightened reactivity and difficulty feeling safe or regulated. Client appears to be beginning to understand how these patterns manifest in her present functioning, though further exploration is needed to fully process and integrate these experiences. Client remains appropriate for current level of care as she continues to require therapeutic support in developing emotional awareness, nervous system regulation, and adaptive coping strategies.

Plan:
Continue to support client in exploring childhood experiences and their impact on current emotional and behavioral patterns. Therapist will provide psychoeducation on the nervous system, specifically the fight-or-flight response, to enhance understanding and normalize her experiences. Interventions will focus on building regulation skills, including grounding techniques and mindfulness practices, to decrease reactivity and increase emotional stability. Encourage client to identify triggers and early warning signs of dysregulation. Follow up in next session to continue processing these connections and assess progress in implementing coping strategies.

Select the dimension of ASAM

- ☐ Dimension 1: Acute Info
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☐ Dimension 6: Recovering/Living Environment

Staff

Marleigh Johnson, MS, LPC
06/12/2026 1:44PM

06/12/2026

Appt. With	Title	Topic	Date
Tori Barranco, BSW	Group - Friday AM	Group Therapy	06/12/2026

Add-On Service

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Duration	Status	Group Attendees	Session Times
180 Min	Attended	8	9:00 AM - 12:00 PM

Treatment Objectives

will identify emotional triggers and physiological signs of distress during group sessions.

Group Notes
Today's group focused on the concept of recovery capital and its role in supporting long-term recovery. The session began with a check-in, allowing participants to discuss current challenges, successes, and recovery-related experiences. This was followed by psychoeducation on recovery capital, including an overview of its various dimensions and how personal, social, community, and cultural resources can influence recovery outcomes. Participants then completed a Recovery Capital Assessment to identify their individual strengths and areas for growth. The group concluded with a discussion of assessment results, helping members identify specific areas to focus on in order to strengthen their recovery capital and support their ongoing recovery goals.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note

Assessment: was engaged in group and participated in discussions, though she was occasionally distracted. She identified her spiritual well-being as a significant strength and source of motivation in her recovery. also expressed a desire to develop additional emotional regulation skills to help improve her relationships and navigate interpersonal challenges more effectively.

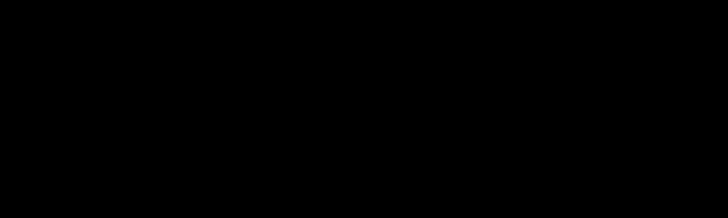
Fetching Report ...

Plan: Encourage her to build on her spiritual strengths as a source of support in recovery while exploring how these strengths can be applied to interpersonal situations. Monitor her engagement in group and provide opportunities to practice healthy communication and coping strategies to improve relationship functioning.

Select the dimension of ASAM

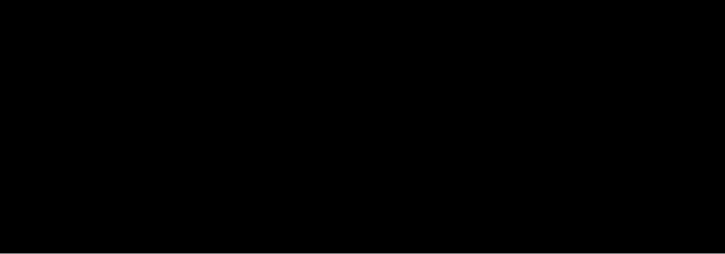
- ☐ Dimension 1: Acute Into
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☐ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff



Tori Barranco, BSW
06/15/2026 8:38AM

Reviewer



Marleigh Johnson, MS, LPC
06/15/2026 1:44PM

06/12/2026

Appt. With	Title	Topic	Date
Saffron Brocious, BA	Group 1 - Friday AM	Group Therapy	06/12/2026

Add-On Service
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Duration	Status	Group Attendees	Session Times
180 Min	Attended	22	9:00 AM - 12:00 PM

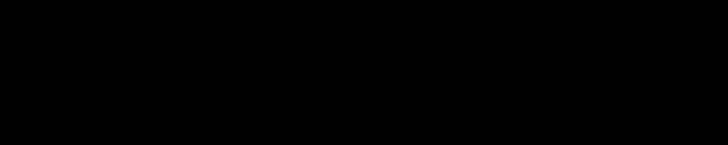
Treatment Objectives
-- No Treatment Objectives is provided --

Group Notes
Clients were given the opportunity to check in and share any updates or challenges they want to discuss with the group. They were prompted to reflect on an emotion that has been particularly difficult for them to handle. We discussed cross/replacement addictions. Facilitator provided psychoeducation about cross addictions and why they often show up in recovery from drugs and alcohol. Clients shared their experiences with replacement addictions that show up for them and triggers that may lead to these behaviors. Clients were then broken into triads and did an activity where they were given a brief case study outlining a person struggling with a cross addiction. They had to identify the addiction, the reason the person may be engaging in it, the emotions they may be struggling with, and come up with a mini treatment plan with goals for the person.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note
-- No assessment note is provided. --

Staff



Saffron Brocious, BA
06/16/2026 9:41AM

Fetching Report ...

Date
06/15/2026

—

Session Times
9:00 AM - 12:00 PM

-- No Treatment Objectives is provided --

-- No group note is provided --

-- No patient exercise note is provided --

-- No assessment note is provided. --

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06/16/2026

Date
06/16/2026

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Session Times
9:00 AM - 12:00 PM

- # [REDACTED] will identify emotional triggers and physiological signs of distress during group sessions.
- # [REDACTED] will identify triggers for impulsive decisions and substance use during group and individual therapy.
- # [REDACTED] will learn and apply assertive communication skills in group sessions.

Data:

Facilitator led a process group focused on the concept of time in recovery. Discussion centered on members' expectations for rapid change versus the reality that recovery is often gradual and requires sustained effort. Facilitator provided psychoeducation on the importance of patience, emphasizing that meaningful behavioral and cognitive change typically occurs over extended periods rather than on an individual's preferred timeline. Group explored the role of consistent, small actions (e.g., daily routines, coping skill practice, attendance in treatment and support systems) in creating long-term progress. Members were encouraged to reflect on personal experiences of frustration with the pace of recovery and to identify areas where they may benefit from increased patience and persistence. Engagement was observed through active listening, verbal participation, and shared reflections related to challenges with delayed gratification and maintaining motivation over time.

-- No patient exercise note is provided --

Assessment:
Member presents with increased resilience and improved emotional regulation despite exposure to high-risk triggers and interpersonal conflict. Recent return to home environment resulted in contact with a peer associated with past substance use, including direct exposure to a drug dealer, representing a significant relapse risk. Member additionally experienced interpersonal criticism via text, which historically would have escalated emotional distress. However, member demonstrates notable progress in self-esteem and emotional stability, as evidenced by ability to internalize less of the criticism and avoid spiraling. Member also reports experiencing a panic attack but successfully utilized self-soothing techniques to regulate emotions, marking a significant improvement in coping capacity. Member's report that the panic symptoms resolved without lingering physical distress further supports progress in managing anxiety. Overall, while relapse risk remains present due to environmental triggers, member is demonstrating increased use of adaptive coping skills and improved distress tolerance.

Plan:

Continue to reinforce and validate member's effective use of coping strategies, particularly self-soothing and emotional regulation during panic symptoms. Support continued development of relapse prevention strategies specific to high-risk environments and individuals, including maintaining boundaries with peers associated with substance use. Encourage ongoing processing of interpersonal conflict and strengthening of self-concept to reduce vulnerability to external criticism. Provide psychoeducation on panic management and reinforce skills that contributed to successful symptom resolution. Monitor for continued exposure to triggers and assess impact on cravings and emotional stability. Encourage continued participation in group to build insight and reinforce progress while further developing resilience and coping skills.

Select the dimension of ASAM

- ☐ Dimension 1: Acute Intoxication and/or Withdrawal Potential
- ☐ Dimension 2: Biomedical Conditions and Complication
- ☒ Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications
- ☒ Dimension 4: Readiness to Change
- ☐ Dimension 5: Relapse, Continued Use or Continued Problem Potential
- ☒ Dimension 6: Recovering/Living Environment

Staff [Redacted]

Caleb Madison, BS
06/16/2026 12:45PM

06/16/2026

Appt. With	Title	Topic	Date
Marleigh Johnson, MS, LPC	Group 1 - Tuesday AM	Group Therapy	06/16/2026

Add-On Service
--

Duration	Status	Group Attendees	Session Times
180 Min	Incomplete	14	9:00 AM - 12:00 PM

Treatment Objectives
-- No Treatment Objectives is provided --

Group Notes
The clinician facilitated a process group utilizing structured check-in questions to encourage reflection and engagement. Following check-in, the group transitioned into an open discussion centered on trust and self-trust. Participants explored their personal experiences with trusting others and examined challenges related to trusting their own judgment, particularly in the context of recovery. Group members shared how past experiences, including active substance use, have impacted their ability to trust themselves and others. The clinician guided the discussion to highlight connections between trust, decision-making, vulnerability, and relationship patterns. Psychoeducation was provided on the role of self-trust in recovery, emphasizing the importance of rebuilding confidence in one's choices and developing consistent, healthy behaviors. The group environment remained supportive, with members offering feedback, validation, and shared experiences.

Patient Exercise
-- No patient exercise note is provided --

Assessment Note
-- No assessment note is provided. --

Staff [Redacted]

Marleigh Johnson, MS, LPC
06/16/2026 11:35AM

TRANSPORT APPOINTMENT

-- No Transport Appointments --

Fetching Report ...

EMERGENCY CONTACTS

Name	Email				te
Junction House		(215) 815-011	Yes	05/29/2026	05/28/2027
Sunlight Wellness	dromero@sunlightwellnesspa.com	(267) 333-6738	Yes	05/29/2026	05/28/2027
Porchlight Recovery		(833) 799-6500	Yes	05/05/2026	05/04/2027
Alex Da Rocha		(240) 888-2710	Yes	05/01/2026	04/30/2027
Alessandra		(240) 449-0759	Yes	05/01/2026	04/30/2027

Fetching Report ...

INTAKE: ADMISSIONS

Reason for Seeking Treatment

Reason for Seeking Treatment

Reason for seeking treatment?
I had a raging addiction to cocaine and alcohol. I used so much I thought I would die and my family got me to go to Caron. I am coming here for aftercare.

What is your primary drug of choice?
Cocaine and Alcohol

Describe current cravings and how they are managed:
Not really. Mostly a want but not a need. I spend a lot of time thinking about using and how I will eventually use at somepoint in my life.

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Cocaine	0	snorting	2 grams	03/07/2026	22	2	getting it from a dealer	Continuous
Alcohol	0	Drinking	Bottle of wine	03/06/2026	16	8	buy it	Continuous

Previous Treatments

TX Center Name	Duration	Level Of Care	What was the response to previous treatment? How long did you stay sober after discharge?
Caron	60	3.5 and 2.5	I am still sobe now. I just left yesterday.

Consequences of Use - BPSA

Have you ever had a blackout?

YesNo

Have you ever tried to hide your substance use from others?

YesNo

Have you ever tried to cut down or stop your substance use?

YesNo

Have you ever used substances in the morning?

YesNo

Fetching Report ...

Do you feel you have a problem with substances?

☐

Yes

☐

No

Have you ever driven while under the influence of substances?

☐

Yes

☐

No

Have you ever lost friends or relationships because of substance use?

☐

Yes

☐

No

Has anyone ever said they were concerned about your substance use?

☐

Yes

☐

No

Have you decreased your social/recreation activities because of substances?

☐

Yes

☐

No

Have you ever felt bad or guilty about your substance use?

☐

Yes

☐

No

What negative consequences have you experienced due to drug or alcohol abuse?

Lost my job. I was a teacher. I never used in school. I worked with babies and 3-6 years old. I lost the job because I was always calling out sick because of my use. I started dating a guy who is an alcoholic. We would use together and he enabled me.

Priority Population Screening Questions*

Priority Population Screening Questions

(The following questions help us determine urgency of admission and ensure timely access to care)

Pregnancy & Substance Use

Are you currently pregnant?

☐

Yes

☐

No

☐

Unsure

If Yes, are you currently using any substances (including alcohol or drugs)?

☐

Yes

☐

No

If Yes, have you injected any substances within the past year?

☐

Yes

☐

No

Injection Drug Use

Have you injected any drugs within the past year?

☐

Yes

☐

No

Overdose History

Fetching Report ...

Have you experienced an overdose within the past 12 months?

☐ Yes ☐ No

If Yes, approximately when did the overdose occur?

Veteran Status

Have you ever served in the United States Armed Forces?

☐ Yes ☐ No

Notes/Comments

TB Risk Screening Questions*

TB Risk Screening Questions

Have you traveled extensively (more than 4 weeks) outside the U.S. in the last five years to high TB-incidence areas (Asia, Africa, South America, Central America)?

☐ Yes ☐ No

Are you an immigrant from a high TB-risk foreign country (includes countries in Asia, Africa, South America, and Central America)?

☐ Yes ☐ No

Have you resided in any of these facilities in the past year: jails, prisons, shelters, nursing homes and other long-term care facilities such as rehabilitation centers?

☐ Yes ☐ No

Have you had any close contact with someone diagnosed with TB?

☐ Yes ☐ No

Have you been homeless within the past year?

☐ Yes ☐ No

Have you ever injected drugs?

☐ Yes ☐ No

Do you or anyone in your household currently have the following symptoms, such as a sustained cough for two or more weeks, coughing up blood, fever/chills, loss of appetite, unexplained weight loss, fatigue, night sweats?

☐ Yes ☐ No

Do you currently have or anticipate having any condition that would decrease your immune system? (Examples: HIV infection, organ transplant recipient, treatment with TNF-alpha antagonist (e.g. infliximab, etanercept, others), steroids (equivalent dose of Prednisone 15mg/day for one month or longer) or any other immunosuppressive medications)

☐ Yes ☐ No

Would you like a referral to a TB clinic for support or testing?

☐ Yes ☐ No

Fetching Report ...

Notes/Comments

Contagious Disease Screening*

Contagious Disease Screening

Hepatitis Status

Have you ever been tested for Hepatitis B?

☐ Yes ☐ No ☐ Unsure

If Yes, what were the results?

☐ Positive ☐ Negative ☐ Unsure

Have you ever been tested for Hepatitis C?

☐ Yes ☐ No ☐ Unsure

If Yes, what were the results?

☐ Positive ☐ Negative ☐ Unsure

Injection Drug Use

Have you ever injected drugs (used a needle to inject any substance), even one time?

☐ Yes, currently ☐ Yes, in the past ☐ No

Intranasal Drug Use

Have you ever used drugs intranasally (snorted substances such as cocaine, heroin, fentanyl, etc.)?

☐ Yes, currently ☐ Yes, in the past ☐ No

HIV Status

Have you ever been diagnosed with HIV?

☐ Yes ☐ No ☐ Prefer not to answer

If Yes, are you currently receiving medical care for HIV?

☐ Yes ☐ No

Have you had sexual contact with:

- ☐ A person living with HIV
- ☐ A person whose HIV status you did not know
- ☐ A person who has multiple sexual partners
- ☐ None of the above

Do you currently have multiple sexual partners?

Fetching Report ...

☐ Yes ☐ No

Have you ever shared needles, syringes, or other drug preparation equipment with another person?

☐ Yes ☐ No

Have you ever been diagnosed with viral hepatitis (Hepatitis A, B, or C) or a sexually transmitted infection?

☐ Yes ☐ No ☐ Unsure

Incarceration History

Have you ever been incarcerated (jail, prison, detention facility)?

☐ Yes ☐ No

Referral and Education

Would you like assistance with any of the following services?

- ☐ Referral for Viral Hepatitis testing
- ☐ Referral for HIV testing
- ☐ Education about Hepatitis or HIV prevention and treatment
- ☐ Support services related to Hepatitis or HIV care
- ☐ Not at this time

Notes/Comments

Suicide/Homicidal Risk Screen - BPSA

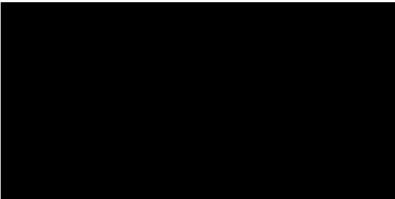
Have you ever experienced HI or SI or Self Harm? If yes, please explain. Include plans, dates, methods, lethality.

No HI. Since I was young I struggled with SI. I have thoughts about disappearing. I am now scared of dying. I didn't use to be. When I was 17- 19 I was the worst. I never made an attempt. I got close. I used to self harm a lot. I stopped for three years and I started again. When I was at Caron I used once.

Do you have any current HI or SI or Self Harm? If yes, please explain.

I haven't had any real SI recently. I sometimes feel like I don't want to be here anymore but I am scared of dying. I love my mother and I wouldn't do that to her. The last time I did cutting was over a month ago.

Digitally Signed by



Caleb Madison, BS 05/01/2026 12:29PM

Fetching Report ...

Reviewer [REDACTED]



Marleigh Johnson, MS, LPC 05/04/2026 11:31AM

----- END OF ADMISSIONS INTAKE -----

Fetching Report ...

INTAKE: FACE TO FACE

Other Information

First Contact Date:	05/01/2026	Prospective Client Intake By:	Ethan Jandrew, BA
Referral Company :		Referral Contact :	
Admission Date :	05/01/2026	Discharge/Transfer:	
Facility Name :	R3 Recovery Services	Prospective Client Status :	Active
Primary Clinician :	Marleigh Johnson	Prospect Status :	Anticipated Admission
Level of care :	IOP 5	Treatment :	

ROI Contacts

Name	Relation	City	State	Country	Mobile	ROI	Emergency Contact
Alessandra [REDACTED]	Mother	Maryland	Pennsylvania	United States	(240) 449-0759	Yes	Yes
Alex Da Rocha	Father				(240) 888-2710	Yes	No
Porchlight Recovery	Other	Lancaster	Pennsylvania	United States	(833) 799-6500	Yes	No
Sunlight Wellness	Other	Chalfont	Pennsylvania	United States	(267) 333-6738	Yes	No
Junction House	Other	Warminster	Pennsylvania		(215) 815-011	Yes	No

Insurance

Insurance	Subscriber	Relationship	Plan	Policy No.	Priority	Co-Pay	Status
United Health Care	[REDACTED]	Self	None Specified	51695708102	Primary		Active

Digitally Signed by



Caleb Madison, BS 05/01/2026 12:32PM

Reviewer



Marleigh Johnson, MS, LPC 05/04/2026 11:31AM

----- END OF FACE TO FACE INTAKE -----
Fetching Report ...

INTAKE: BIO PSYCHO SOCIAL

Reason for Seeking Treatment

Reason for Seeking Treatment

Reason for seeking treatment?

I had an addiction to cocaine and alcohol. I used so much I thought I would die and my family got me to go to Caron. I am coming here for aftercare.

What is your primary drug of choice?

Cocaine and Alcohol

Describe current cravings and how they are managed:

Not really. Mostly a want but not a need. I spend a lot of time thinking about using and how I will eventually use at somepoint in my life.

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Cocaine	0	snorting	2 grams	03/07/2026	22	2	getting it from a dealer	Continuous
Alcohol	0	Drinking	Bottle of wine	03/06/2026	16	8	buy it	Continuous

Treatment and Recovery History - BPSA

Have you ever been treated for substance abuse?

☒ Yes ☐ No

Facility	Name of Provider	ASAM LOC	Approximate Date	Duration	Outcome
Caron		3.5, 2.5	2026-03-07	60 days	Completed
			Fetching Report ...		

Have you ever attended a meeting for AA or NA? ☒ Yes ☐ No

Have you ever had a sponsor? ☒ Yes ☐ No

Longest length of sobriety? 58 days. right now!

How did you remain sober?
I was at Caron. I had a lot of structure

Why did you relapse?
None

Other addictive behaviors:
Shopping. I spend a lot of money on urban outfitters and amazon. I have no money because of it. I have been trying to limit my spending but I don't have much control over what I spend.

Describe your strengths that will help you with recovery:
I am strong. I am very aware of what I shouldn't be doing.

Describe your challenges or barriers to recovery:
Future stuff. Feeling like I will never have fun again. Bad thoughts. I fight with if I can use responsibly.

Fetching Report ...

Gambling

Gambling

Do you gamble? If yes, what type and how often?
None

Do you feel unable to control how often you gamble?
None

Do you feel unable to control how much money you bet?
None

Do you lie or minimize your gambling to others?
None

Has your gambling caused any financial, relationship, or work problems?
None

Consequences of Use - BPSA

Have you ever had a blackout? ☐ Yes ☐ No

Have you ever tried to hide your substance use from others? ☐ Yes ☐ No

Have you ever tried to cut down or stop your substance use? ☐ Yes ☐ No

Have you ever used substances in the morning? ☐ Yes ☐ No

Do you feel you have a problem with substances? ☐ Yes ☐ No

Have you ever driven while under the influence of substances? ☐ Yes ☐ No

Have you ever lost friends or relationships because of substance use? ☐ Yes ☐ No

Has anyone ever said they were concerned about your substance use? ☐ Yes ☐ No

Fetching Report ...

Name	Age	Sex	Quality of Relationship	In your custody?

☐ Check here if no children

How has your relationship with family/significant other been affected by your substance use?

They don't trust me as much. Thye are very supportive. I feel like I fucked things up bad. I spent a lot of their money and I spend a lot of their money. I don't have a car so I am using my dad's car. There is a lot of walking on egg shells from both sides.

Would you like to have your family involved in your treatment?

I would love to do family sessions with them. My mom recently started going to alenon meetings.

My step dad has a hard head.

Describe your current living environment and how it affects your recovery:

I am living at porch light. I got ride of my apartment and I am not planning to go back there.

Family History - BPSA

Has anyone in your family ever been diagnosed with a mental health condition?

☒ Yes ☐ No

*If yes, fill in the blanks below

Family Member	Diagnosis	Type of Treatment

Does anyone in your family have a history of substance/alcohol abuse?

☒ Yes ☐ No

*If yes, fill in blanks below

Fetching Report ...

Family Member	Type of Drug	Outcome

Does anyone in your family have a serious illness or chronic medical condition?

☐ Yes ☐ No

*If yes, fill in blanks below

Family Member	Diagnosis

Please describe your home environment as a child

I was 5 when my parents seperated. I was 12 when my mom got married to my step dad they have been together since I was 6.

My dad had a lot of different relationships but he has been in a relationship with the same women for 8 years. My dad got arrested when I was 1 and he got out when I was 4 or 5. He was a cop and was getting dirty money.

I lived with my mom mainly. My dad would dump a lot on me emotionally and blame my mom for stuff and make me carry that. He would talk badly about my step dad. I would resent my mom too so I ended up spending a lot of time with my dad.

Please describe yourself as a child

I was loud and all over the place. I loved telling stories and making videos and getting my picture taken. I loved music and art. I used to do story time with my stuffed animals.

Describe your current relationship with your parents

Mom - good. We have a lot to work on but it's good. She is a little controlling. I don't tell her a lot because I don't want her to feel stressed or worried.

Step Father - It's bad. We aren't close at all. He is just a guy in my life.

Father - We are distant. I don't think about him a lot.

Step mom - she is sweet and I like her.

Fetching Report ...

Facility Name	Location	Therapist	Diagnosis/Medication	Reason for Treatment	Response to Treatment/ Intervention	Date

Are you currently experiencing any mental health symptoms?

☒ Yes ☒ No

Please elaborate on current symptoms and how your daily functioning is impacted:

I feel like I deal with it every day. I feel like I am not in control of my feelings emotions and thoughts.

Do you use drugs or alcohol to relieve mental health symptoms?

☒ Yes ☒ No ☒ NA

Are you currently seeing a psychiatrist?

☒ Yes ☒ No

If so, what is their name and when is your next scheduled appointment?

I have a list of people that I need to reach out to .

Suicide/Homicidal Risk Screen - BPSA

Have you ever experienced HI or SI or Self Harm? If yes, please explain. Include plans, dates, methods, lethality.

No HI. Since I was young I struggled with SI. I have thoughts about disappearing. I am now scared of dying. I didn't use to be. When I was 17- 19 I was the worst. I never made an attempt. I got close. I used to self harm a lot. I stopped for three years and I started again. When I was at Caron I used once.

Do you have any current HI or SI or Self Harm? If yes, please explain.

I haven't had any real SI recently. I sometimes feel like I don't want to be here anymore but I am scared of dying. I love my mother and I wouldn't do that to her. The last time I did cutting was over a month ago.

C-SSRS Screener (Safety Plan for mod/high risk) Outpatient

COLUMBIA-SUICIDE SEVERITY RATING SCALE
Screen Version - Recent

Suicide Ideation Definitions and Prompts

Fetching Report ...

Ask Questions 1 and 2

Month

1) Wish to be dead

Have you wished you were dead or wished you could go to sleep and not wake up?

☐ Yes
☐ No

2) Current suicidal thoughts

Have you actually had any thoughts of killing yourself?

☐ Yes
☐ No

If YES to 2, ask questions 3, 4, 5, and 6. If NO to 2, go directly to question 6.

Month

3) Suicidal thoughts w/ Method (w/no specific Plan or Intent or act)

Have you been thinking about how you might do this?

☐ Yes
☐ No

4) Suicidal Intent without Specific Plan

Have you had these thoughts and had some intention of acting on them?

☐ Yes
☐ No

5) Intent with Plan

Have you started to work out or worked out the details of how to kill yourself? Do you intend to carry out this plan?

☐ Yes
☐ No

6) C-SSRS Suicidal Behavior: *"Have you ever done anything, started to do anything, or prepared to do anything to end your life?"*

Examples: Collected pills, obtained a gun, gave away valuables, wrote a will or suicide note, took out pills but didn't swallow any, held a gun but changed your mind or it was grabbed from your hand, went to the roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Lifetime

☐ Yes
☐ No

If "YES" Was it within the past 3 months?

Past 3 Months

☐ Yes
☐ No

Fetching Report ...

C-SSRs Score:

High

Moderate

Low

No Identified Risk

Comments:

Columbia-Suicide Severity Rating Scale

Intensive Outpatient Programming/ Outpatient Programming: Interventions for each risk level may include the following. Risk level and interventions must always be documented in the electronic health record.

High	Referral to local emergency room Referral to crisis team Referral and evaluation by a mental health professional or psychiatry (internal or external) Re-evaluate for level of care; reassess at treatment plan review for continuing in program Engagement with family/significant other Personal safety plan Create/update IAPP Give crisis team/suicide risk number at discharge
Moderate	Referral and evaluation by a mental health professional or psychiatry (internal or external) Engagement with family/significant other Personal safety plan Create/update IAPP Give crisis team/suicide risk number at discharge Reassess at treatment plan review
Low	Mental health resources and support provided - internal or external Give crisis team/suicide risk number at discharge Provide information about suicide risk warning signs Re-assess at treatment plan review

Trauma Screening - BPSA

Were you the victim of any Trauma, Abuse, Neglect or Exploitation? If yes, please provide details

Yes. Abusive ex-boyfriend. This was when I was 17- 19

Have there been any traumatic events(that you have witnessed or experienced) that you feel have affected you over the course of your life?

None

Have you been the perpetrator of abuse in the past? If so please provide details and dates.

None

Fetching Report ...

Medical - BPSA

Do you have any medical problems? Including dental problems

☐ Yes ☐ No

If yes, please explain

Are you currently under the care of a medical doctor?

☐ Yes ☐ No

Please provide medical doctors name and when your last physical exam was

Kathleen Adeshoga, MD 3 months ago

Do you have any life-threatening allergies?

☐ Yes ☐ No

Explain adverse reactions. Do you have a rescue medication with you?

Do you have any communicable disease(s)?

☐ Yes ☐ No

Explain:

Do you have any chronic medical conditions?

☐ Yes ☐ No

Condition	Date of Onset	Current treatment Provider	Current intervention and response to intervention

MEDICATION WILL BE FOUND IN THE RX BUBBLE ON THE CLIENT CHART

Have you had any major injuries or surgeries?

☐ Yes ☐ No

Fetching Report ...

Date	Location	Reason	Outcome

Do you have any food allergies? Explain adverse reactions.
None

Do you have any drug allergies? Explain adverse reactions.
None

Educational Level - BPSA

Highest Grade/Degree Completed:
Highschool and two years of College

Do you have any diagnosed learning disabilities including attention deficit hyperactivity disorders?
☒ Yes ☒ No

Explain:

Does this affect you today?
☒ Yes ☒ No

How?

Do you have current academic goals?
I want to go back to school to get my degree in early ed, marine biology.

Vocational Screen - BPSA

Are you currently employed?
☒ Yes ☒ No

Explain:
Bar tender. I stopped working there when I went to treatment.

Fetching Report ...

Do you need assistance developing job skills or find a job?

☐ Yes

☐ No

Have you ever missed work or lost a job due to your substance use?

☐ Yes

☐ No

Explain:

I got fired from my first job

What is your current financial status?

☐ Self Sufficient

☐ Supported by Family

☐ Struggling

Military History - BPSA

Have you ever served in the military?

☐ Yes

☐ No

If yes, which branch?

Dates served:

Recreation/ Leisure Activities - BPSA

What do you do for social or leisure activities and how can it support your recovery? Pleast list any current support groups or social services that are currently being utilized.

Art, music, hangout with friends, going blue marsh lake. I love being outdoors

Describe how substance use has impacted this area:

I don't do anything but use.

Legal History - BPSA

History of Legal Problems?

☐ Yes

☐ No

Date	Charge	Outcome	State

Fetching Report ...

Date	Charge	Outcome	State

Do you have any criminal charges pending?

☐ Yes ☐ No

Explain:

Are you currently on parole or probation?

☐ Yes ☐ No

What agency?

Do you have a valid driver's license?

☐ Yes ☐ No

Do you have transportation needs?

☐ Yes ☐ No

Explain:

Gender/ Sexual Orientation and sexual functioning - BPSA

Orientation

Bisexual

Have you put yourself in any risky sexual situations?

☐ Yes ☐ No

If yes, Please explain

Do you have any guilt or shame regarding your sexual orientation or sexual behavior?

None

Spiritual/ Religious Assessment - BPSA

Religious/ Spiritual Preference:

Non-religious

Practicing?

☐ Yes ☐ No

What is your history with spirituality?

Fetching Report ...

Raised catholic. I am exploring different things

What kind of spiritual activities do you participate in and how does it affect your recovery?

I believe in God to an extent. I am exploring different things. I am spirtual.

Culture - BPSA

Ethnic background, cultural beliefs and practices:

I am from Brizil

SNAG/SNAP

Strengths : I care a lot about people. I am very empathetic. I give good advice.. I am very smart and kind.

Needs : I want to use those strengths on myself. Setting boundries with myself. I give all of my self to others and I get lost.

Abilities : I can sing, dance, play guitar and piano.

Goals (Preferences) : I want to be okay with who I am. I don't want to always be chasing the next best thing.

Generalized Anxiety Disorder 7-item (GAD-7) Scale

GAD-7

Over the last 2 weeks, how often have you been bothered by the following problems?

1. Feeling nervous, anxious or on edge

☐ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

2. Not being able to stop or control worrying

☐ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

3. Worrying too much about different things

☐ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

4. Trouble relaxing

☐ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

5. Being so restless that it is hard to sit still

☐ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

6. Becoming easily annoyed or irritable

☐ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

7. Feeling afraid as if something awful might happen

☐ Not at all ☐ Several days ☐ More than half the days ☐ Nearly every day

GAD7 Score:

21

If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

☐ Not difficult at all ☐ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Fetching Report ...

Patient Health Questionnaire (PHQ-9)

PATIENT HEALTH QUESTIONNAIRE (PHQ-9)

Over the last 2 weeks, how often have you been bothered by any of the following problems?

1. Little interest or pleasure in doing things.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

2. Feeling down, depressed, or hopeless.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

3. Trouble falling asleep, staying asleep, or sleeping too much.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

4. Feeling tired or having little energy.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

5. Poor appetite or overeating.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

6. Feeling bad about yourself - or that you are a failure or have let yourself or your family down

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

7. Trouble concentrating on things such as reading the newspaper or watching television.

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

8. Moving or speaking so slowly that other people could have noticed? Or the opposite - being so fidgety or restless that you have been moving around a lot more than usual

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

9. Thoughts you would be better off dead or of hurting yourself in some way

☐ Not at all ☐ Several Days ☐ More than half the days ☐ Nearly every day

PHQ-9 Score:

22

10. If you checked off any problems, how difficult have these problems made it for you to do your work, take care of things at home, or get along with other people?

☐ Not difficult at all ☐ Somewhat difficult ☐ Very difficult ☐ Extremely difficult

Brief Assessment of Recovery Capital (BARC-10)

Brief Assessment of Recovery Capital (BARC-10)

Instructions: On a scale of 1 (Strongly disagree) to 6 (Strongly agree), please indicate your level of agreement with the following statements.

1. There are important things to me in life than using substances.

☐ 1 Strongly Disagree ☐ 2 Disagree ☐ Somewhat Disagree ☐ 4 Somewhat Agree ☐ 5 Agree ☐ 6 Strongly Agree

2. In general I am happy with my life.

☐ 1 Strongly Disagree ☐ 2 Disagree ☐ Somewhat Disagree ☐ 4 Somewhat Agree ... ☐ 5 Agree ☐ 6 Strongly Agree

3. I have enough energy to complete the tasks I set myself.

1 Strongly Disagree	2 Disagree	3 Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree
4. I am proud of the community I live in and feel part of it.					
1 Strongly Disagree	2 Disagree	Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree
5. I get lots of support from friends.					
1 Strongly Disagree	2 Disagree	Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree
6. I regard my life as challenging and fulfilling without the need for using drugs or alcohol.					
1 Strongly Disagree	2 Disagree	Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree
7. My living space has helped to drive my recovery journey.					
1 Strongly Disagree	2 Disagree	Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree
8. I take full responsibility for my actions.					
1 Strongly Disagree	2 Disagree	Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree
9. I am happy dealing with a range of professional people.					
1 Strongly Disagree	2 Disagree	Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree
10. I am making good progress on my recovery journey.					
1 Strongly Disagree	2 Disagree	Somewhat Disagree	4 Somewhat Agree	5 Agree	6 Strongly Agree

Scoring

Total scores can range from a miinimm of 10 to a maximum of 60. A ROC Curve Analysis showed the BARC-10 had predictive validity with sustained remission (i.e., 1 year or more) using a cut-off score of 47 in a sample whose average length of recovery time was 7 years.

Diagnosis

Diagnostic Impression	Priority	Created By	Created Date	Resolved
[F14.20] Cocaine use disorder, Severe	Primary	Caleb Madison	05/01/2026	--
[F10.20] Alcohol use disorder, Severe	Secondary	Caleb Madison	05/01/2026	--

Mental Status Summary

Hygiene

WNL(within normal limit)	Clean	Unshaved
Odorous	Disheveled	Other

Comments:

Affect

WNL(within normal limit)	Appropriate	Inappropriate
Labile	Blunted	Flat
Restricted	Expansive	Euphoric
Anxious	Angry	Other

Comments:

Hallucinations

WNL(within normal limit)	Visual	Auditory
Tactile	Olfactory	Command
Denied	Other	

Comments:

Movement

WNL(within normal limit)	Normal	Tense
Fidgety	Hyperactive	Psychomotor Retardation
Psychomotor Agitation	Pacing	Hand Wringing
Tics	Other	

Comments:

Mood

WNL(within normal limit)	Anxious	Confused
---	--	---

Fetching Report ...

Depressed
Euphoric
Irritated
Other

Comments:

Speech

WNL(within normal limit)
Loud
Slowed

Comments:

Impulse Control

WNL(within normal limit)
Poor

Comments:

Insight

WNL(within normal limit)
Poor

Comments:

Intelligence

WNL(within normal limit)
Below Average

Comments:

Oriented

WNL(within normal limit)
Below Average

Comments:

Other

WNL(within normal limit)
Depersonalization

Comments:

Memory

WNL(within normal limit)
ShortTermImpaired

Comments:

Suicidal Ideation

WNL(within normal limit)
Intent

Comments:

Homicidal Ideation

WNL(within normal limit)
Intent

Comments:

Eye Contact

WNL(within normal limit)
Fleeting

Comments:

Facial Expression

WNL(within normal limit)
Angry
Sad

Comments:

Thought Process

WNL(within normal limit)

Labile
Angry
Euthymic

Normal
Pressured
Slurred

Good

Good

Above Average

Person
Situation

Obsessions
DeRealism

Intact
LongTermImpaired

Yes
Means

Yes
Means

Good
Stare

Amiable
Worried

Intact

Fearful
Dysthymic
Normal

Soft
Rapid
Incoherent

Limited

Limited

Average

Place

Compulsions
Denied

ImmediateImpaired

Plan
Denied

Plan
Denied

Avoidant

Happy
Suspicious

Blocked

Fetching Report ...

Loose

Flight

Comments:

WNL(within normal limit)

Somatic

Denied

Delusions

Grandiose

Paranoia

Other

Comments:

WNL(within normal limit)

Guarded

Friendly

Demanding

Worried

Seductive

Other

Demeanor

Cooperative

Resistive

Worried

Friendly

Evasive

Arrogant

Comments:

WNL(within normal limit)

Easily Distracted

Attention/Concentration

Satisfactory

Other

Comments:

WNL(within normal limit)

Poor

Judgment

Good

Other

Comments:

Tangential

Other

Comments:

Persecutory Ideas

Comments:

Uncooperative

Defensive

Boastful

Irritable

Dramatic

Apathetic

Comments:

Poor

Comments:

Limited

Comments:

ASAM Placement

ASAM Placement Summary Sheet

Date:

2026-05-01

Type (Check One):

Admission

Continued Stay

Discharge

Dimension

	Level of Risk	Level of Care	Criteria Included/Comments
D1. Acute intoxication and/or withdrawal potential:	0	2.5	Andrade is not currently experiencing acute intoxication or withdrawal symptoms. She reports last use of alcohol on 03/06/2026 and cocaine on 03/07/2026, following which she completed residential treatment and has remained abstinent. She denies current physical withdrawal symptoms and does not exhibit signs of intoxication or physiological instability. However, given the severity of her historical substance use, including prolonged alcohol use beginning in adolescence and frequent, high-risk cocaine use, continued monitoring remains clinically appropriate. While Dimension 1 risk is currently low, PHP

Fetching Report ...

	Level of Risk	Level of Care	Criteria Included/Comments
			allows for ongoing observation during early recovery to ensure emerging symptoms (e.g., stress-activated cravings or somatic anxiety symptoms) are appropriately managed without escalation. Dimension 1 Summary: There is no immediate withdrawal risk, but continued PHP supports stabilization during early recovery and transition from higher levels of care.
D2. Biomedical Conditions and Complications:	0	2.5	██████ reports no chronic or acute biomedical conditions that would independently interfere with treatment participation. She denies life-threatening medical issues, communicable diseases, allergies, or current medical instability. She is under the care of a primary care physician and reports a physical examination within the past three months. While Dimension 2 needs are currently low, ongoing engagement in PHP ensures coordination of care should somatic concerns arise, particularly given the historical impact of substance use on overall functioning and stress tolerance. Dimension 2 Summary: Biomedical conditions do not currently necessitate a higher level of medical care but are appropriately managed within the PHP setting.
D3. Emotional/Behavioral or cognitive conditions and complications:	2	2.5	██████ presents with significant co-occurring psychiatric conditions that substantially impact her relapse risk and recovery trajectory. She carries diagnoses of Major Depressive Disorder, Generalized Anxiety Disorder, and an unspecified personality disorder. Symptom severity at intake was high, as evidenced by a GAD-7 score of 21 (severe anxiety) and a PHQ-9 score of 22 (severe depression). She reports chronic emotional dysregulation, frequent anxiety, intrusive thoughts, and daily distress, all of which have historically contributed to substance use as a coping mechanism. Additionally, she has a notable history of suicidal ideation and non-suicidal self-injury, with the most recent self-harm episode occurring approximately one month prior to intake. Although she currently denies active suicidal intent or plan, her history and symptom severity indicate elevated clinical risk requiring close monitoring and consistent therapeutic intervention. PHP provides the necessary frequency, intensity, and integration of mental health and substance use treatment, allowing for daily assessment, skills development, and stabilization that cannot be safely managed in a lower level of care at this time. Dimension 3 Summary: The member demonstrates moderate-high psychiatric acuity that significantly interferes with recovery and necessitates continued PHP to prevent decompensation and relapse.
D4. Readiness to change:	2	2.5	██████ demonstrates high motivation and active engagement in her recovery process. She openly acknowledges the severity of her substance use, the risks associated with relapse, and her current limitations in managing cravings and emotional stress independently. Importantly, she has self-advocated for continued PHP, clearly expressing that she does not feel ready to step down due to fear of relapse and emotional destabilization. While she remains motivated, she also exhibits ambivalence related to long-term abstinence, including ongoing thoughts about controlled use and fears about life satisfaction without substances. This ambivalence does not reflect resistance but rather indicates a need for continued motivational enhancement,

Fetching Report ...

	Level of Risk	Level of Care	Criteria Included/Comments
			psychoeducation, and experiential skill-building within a structured setting. PHP supports her readiness to change by matching treatment intensity to clinical need, reinforcing motivation, and helping her internalize recovery tools before transitioning to a less structured environment. Dimension 4 Summary: The member is highly motivated but not yet sufficiently stabilized to safely transition to a lower level of care. Continued PHP appropriately supports her stage of change.
D5. Relapse, continued use or continued problem potential:	3	2.5	██████████ Andrade ██████████ ██████████ presents with a high risk for relapse that warrants continued treatment at the Partial Hospitalization Program (PHP) level of care. Although she is currently abstinent, she reports ongoing and distressing cravings, particularly psychological cravings characterized by persistent thoughts about future substance use and fear of losing control. The member has clearly articulated that her cravings "scare her," indicating awareness of her vulnerability and limited confidence in her ability to manage these urges without significant clinical structure. ██████████ has a recent history of severe cocaine and alcohol use, with escalation leading to perceived risk of death and the need for residential treatment. Her longest period of sobriety (approximately 58 days) has only been achieved within highly structured environments, such as residential treatment and supported housing. She has not yet demonstrated the ability to maintain sobriety in less structured settings. Additionally, she continues to struggle with ambivalence about long-term abstinence, including intrusive thoughts about controlled use, which substantially increases relapse potential. Compounding this risk are significant co-occurring mental health symptoms, including severe anxiety and depression, emotional dysregulation, and a history of self-harm. These conditions have historically contributed to substance use as a maladaptive coping strategy. The member reports that stress, emotional overwhelm, and environmental changes significantly intensify cravings, further elevating relapse risk during transition periods. ██████████ has directly requested to remain at the PHP level of care due to fear of relapse and acknowledges that without intensive structure, daily clinical contact, and accountability, she is unlikely to maintain sobriety. Given her limited recovery capital outside of structured treatment, ongoing cravings, psychological dependence, and history of relapse risk factors, step-down at this time would be clinically premature and places the member at high risk for return to use. Dimension 5 Summary: The member demonstrates high relapse vulnerability without intensive structure. Continued PHP is medically necessary to provide daily therapeutic support, relapse prevention interventions, and monitoring to stabilize cravings and build sustainable coping skills prior to transition to a lower level of care.
D6. Recovery Environment:	3	2.5	ASAM Dimension 6: Recovery Environment ██████████'s current recovery environment significantly supports the need for continued PHP level of care. She reports experiencing high stress related to a major change in setting, including unfamiliar surroundings, lack of local supports, and limited sense of emotional safety in the new area. She has explicitly stated that she has no local social or recovery-based support system and feels isolated and overwhelmed by the transition. The member reports that environmental instability and lack of familiarity exacerbate her anxiety and cravings, increasing risk

Fetching Report ...

	Level of Risk	Level of Care	Criteria Included/Comments
			for relapse. While her family is emotionally supportive, they are not physically present or able to provide daily recovery support in her current location. Additionally, she reports concern that some family members hold misconceptions about addiction and may unintentionally minimize relapse risk, limiting their effectiveness as immediate recovery supports. ██████ has demonstrated insight by recognizing that structure is her primary protective factor. She has proactively advocated for PHP, identifying it as essential to help her adjust to the new environment, establish routines, and develop local recovery supports before transitioning to a less intensive level of care. Without this level of support, she would be navigating early recovery, environmental change, and significant mental health symptoms simultaneously, which places her at high risk for destabilization and relapse. The PHP setting provides the necessary containment, therapeutic consistency, and recovery-focused environment to compensate for current deficits in her external support system. PHP also allows for gradual exposure to independence while maintaining clinical oversight, which aligns with best practices for individuals in early recovery with environmental stressors. Dimension 6 Summary: The member's recovery environment is currently unstable and insufficient to support sustained sobriety without intensive clinical structure. Continued PHP is required to mitigate environmental stressors, provide recovery-oriented containment, and support safe adjustment to her new location.

Indicate level of care recommended:

2.5

Indicate level of care received:

2.5

If recommended level of care is different from received, why?

R3 Recovery Services

Indicate the program or Facility referred to:

N/A

Narrative Summary

Narrative Summary

██████ is a 24-year-old Hispanic female who presented to R3 Recovery Services on 05/01/2026 for aftercare treatment following completion of residential treatment at Caron. She self-referred with strong encouragement from her family after experiencing severe consequences related to substance use. █████ identifies cocaine and alcohol as her primary substances of use and meets criteria for severe Cocaine Use Disorder and severe Alcohol Use Disorder.

██████ began using alcohol at age 16 and reports approximately eight years of problematic use, drinking up to a bottle of wine per episode in a continuous pattern. Cocaine use began at age 22, with use escalating to approximately two grams per episode via intranasal route over a two-year period. Her last reported use of alcohol was 03/06/2026 and cocaine on 03/07/2026. She reports minimal physical cravings at present but endorses persistent psychological preoccupation with substances and ambivalence about long-term sobriety, including ongoing thoughts about potentially returning to controlled use in the future.

██████ completed approximately 60 days of residential treatment at Caron, which she identifies as the longest period of sobriety she has achieved (58 days at intake). She reports sobriety was maintained through intensive structure and external accountability. She denies relapse since discharge and is currently residing in a structured recovery environment (Porch Light), having given up her prior apartment to support sobriety. She demonstrates insight into relapse risk, particularly related to distorted beliefs about responsible use and fears about never enjoying life without substances.

Substance use has resulted in significant functional impairment across multiple domains. Vocationally, █████ lost her job as a teacher due to excessive absenteeism related to substance use. She also reports financial instability, reliance on family support, lack of independent transportation, and compulsive spending behaviors, particularly related to online shopping, which she identifies as another addictive behavior. Interpersonally, substance use has eroded trust within her family system, though she reports her family remains highly supportive and engaged in her recovery. She expresses strong interest in participating in family therapy.

██████ has a significant mental health history, including diagnoses of Major Depressive Disorder, Generalized Anxiety Disorder, and an unspecified personality disorder. She reports chronic emotional dysregulation, daily anxiety, and depressive symptoms that significantly impair functioning. Screening scores indicate severe symptom severity, including a GAD-7 score of 21 and a PHQ-9 score of 22.

██████ reports a longstanding history of suicidal ideation beginning in adolescence, as well as recurrent self-harm (cutting), with the most recent episode occurring approximately one month prior to intake.

Fetching Report ...

While she denies current active suicidal intent or plan, she endorses passive thoughts of not wanting to exist and acknowledges elevated risk, mitigated by protective factors including fear of death, strong attachment to her mother, and active engagement in treatment.

She reports a history of trauma related to an abusive intimate relationship between ages 17 and 19. Family history is notable for instability during childhood, including parental separation, emotional burden placed on her by her father, and inconsistent attachment figures. Despite these challenges, [REDACTED] identifies multiple personal strengths, including empathy, insight, creativity, intelligence, and strong artistic abilities. She enjoys music, art, outdoor activities, and expresses long-term goals of returning to school for early childhood education or marine biology.

Mental Status Examination at intake was within normal limits across all observed domains, including orientation, insight, judgment, thought process, and affect. No psychotic symptoms were observed or reported. Hygiene and behavior were appropriate. Although her current suicide risk is assessed as manageable in an outpatient setting, her history and symptom severity indicate the need for ongoing monitoring, psychiatric evaluation, and structured therapeutic support.

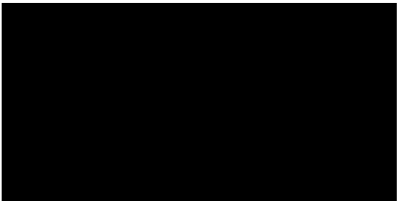
[REDACTED] presents with high motivation for treatment but demonstrates ambivalence regarding lifelong abstinence, placing her at moderate to high relapse risk without continued structure and support. Her recovery environment, family involvement, insight into her substance use, and willingness to engage in services are significant protective factors. Continued treatment focusing on relapse prevention, emotional regulation, trauma processing, cognitive restructuring, and stabilization of mood and anxiety symptoms is clinically indicated.

Digitally Signed by



Caleb Madison, BS 05/04/2026 1:53PM

Reviewer



Marleigh Johnson, MS, LPC 05/06/2026 7:36AM

----- END OF BPS INTAKE -----

Fetching Report ...

INTAKE: MEDICAL - HISTORY & PHYSICAL

Chief Complaint

Present Illness History

Drug Of Choice

Substance Use History: --
Drug of Choice 1: --
Drug of Choice 2: --
Drug of Choice 3: --
Additional substance use history (include withdrawal history and medical complications): --

Mental Health History

Have you ever been given a psychiatric diagnosis? ☐

Have you received mental health services or are you currently receiving services? ☐

Comments : N/A

Past Surgical and Hospitalization History List

-- No Past Surgical or Hospitalization History --

Allergies

-- No Allergies --

Lab Results

Radiology Reports

Assessment

--

Initial Treatment Plan

Medication Orders

Consult Orders

----- END OF MEDICAL INTAKE -----

Fetching Report ...

INTAKE: MEDICAL - NURSING

Vital Signs

--No Vitals --

Allergies

-- No Allergies --

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Cocaine	0	snorting	2 grams	03/07/2026	22	2	getting it from a dealer	Continuous
Alcohol	0	Drinking	Bottle of wine	03/06/2026	16	8	buy it	Continuous

Mental Status Summary

Hygiene

WNL(within normal limit)
Odorous

Clean
Disheveled

Unshaved
Other

Comments:

Affect

WNL(within normal limit)
Labile
Restricted
Anxious

Appropriate
Blunted
Expansive
Angry

Inappropriate
Flat
Euphoric
Other

Comments:

Hallucinations

WNL(within normal limit)
Tactile
Denied

Visual
Olfactory
Other

Auditory
Command

Comments:

Movement

WNL(within normal limit)
Fidgety
Psychomotor Agitation
Tics

Normal
Hyperactive
Pacing
Other

Tense
Psychomotor Retardation
Hand Wringing

Comments:

Mood

WNL(within normal limit)
Depressed
Euphoric
Irritated
Other

Anxious
Labile
Angry
Euthymic

Confused
Fearful
Dysthymic
Normal

Comments:

Speech

WNL(within normal limit)
Loud
Slowed

Normal
Pressured
Slurred

Soft
Rapid
Incoherent

Comments:

Impulse Control

WNL(within normal limit)
Poor

Good

Fetching Report ...

Limited

Comments:

Insight

WNL(within normal limit) Poor	Good	Limited
---	----------------------------	-------------------------------

Comments:

Intelligence

WNL(within normal limit) Below Average	Above Average	Average
--	-------------------------------------	-------------------------------

Comments:

Oriented

WNL(within normal limit) Below Average	Person Situation	Place
--	--	-----------------------------

Comments:

Other

WNL(within normal limit) Depersonalization	Obsessions DeRealism	Compulsions Denied
--	--	--

Comments:

Memory

WNL(within normal limit) ShortTermImpaired	Intact LongTermImpaired	ImmediateImpaired
--	---	---

Comments:

Suicidal Ideation

WNL(within normal limit) Intent	Yes Means	Plan Denied
---	---	---

Comments:

Homicidal Ideation

WNL(within normal limit) Intent	Yes Means	Plan Denied
---	---	---

Comments:

Eye Contact

WNL(within normal limit) Fleeting	Good Stare	Avoidant
---	--	--------------------------------

Comments:

Facial Expression

WNL(within normal limit) Angry Sad	Amiable Worried	Happy Suspicious
--	---	--

Comments:

Thought Process

WNL(within normal limit) Loose Flight	Intact Incoherent Circumstantial	Blocked Tangential Other
---	--	--

Comments:

Delusions

WNL(within normal limit) Somatic Denied	Grandiose Paranoia Other	Persecutory Ideas
---	--	---

Comments:

Demeanor

WNL(within normal limit) Guarded Friendly Demanding	Cooperative Resistive Worried Friendly	Uncooperative Defensive Boastful Irritable
---	--	--

Fetching Report ...

Worried

Seductive

Other

Evasive

Arrogant

Dramatic

Apathetic

Comments:

Attention/Concentration

WNL(within normal limit)

Easily Distracted

Satisfactory

Other

Poor

Comments:

Judgment

WNL(within normal limit)

Poor

Good

Other

Limited

Comments:

----- END OF NURSING INTAKE -----

INTAKE: MEDICAL - PSYCHIATRIC EVALUATION

Substance Use History

Substance Name	Days Used in the last 30	Route	Amount	Last Used Date/Time	Age first used	Years of Use	Method of Acquiring Substance	Pattern of Use
Cocaine	0	snorting	2 grams	03/07/2026	22	2	getting it from a dealer	Continuous
Alcohol	0	Drinking	Bottle of wine	03/06/2026	16	8	buy it	Continuous

COWS Scores

COWS Date	COWS Time	Total COWS Score	Scale Score	Created By	Created Date
-- No COWs --					

CIWA Scores

CIWA Date	CIWA Time	Total CIWA Score	Created By	Created Date
-- No CIWAs --				

CIWA-B Scores

CIWA-B Date	CIWA-B Time	Total CIWA-B Score	Created By	Created Date
-- No CIWA-Bs --				

----- END OF MEDICAL INTAKE -----

Fetching Report ...

INTAKE: BILLING

Insurance

Insurance	Subscriber	Relationship	Plan	Policy No.	Priority	Co-Pay	Status
United Health Care		Self	None Specified	51695708102	Primary		Active

Private Pay

-- No Private Pay --

----- END OF BILLING INTAKE -----

Fetching Report ...

Expected Length of Treatment
(days): **365**

Problem: Substance Use Disorders

Diagnostic Impression:

05/01/2026 - [F14.20] Cocaine use disorder, Severe

05/01/2026 - [F10.20] Alcohol use disorder, Severe

Behavioral Definitions:

05/01/2026 - Continues substance use despite knowledge of experiencing persistent physical, legal, financial, vocational, social, and/or relationship problems that are directly caused by the use of the substance.

Client Goal: Client will increase understanding of addiction, treatment expectations, and program structure to support engagement and successful participation in treatment.

Objective: Client will attend all scheduled appointments, actively participate in individual and group sessions, and meet with their primary therapist to collaboratively develop a comprehensive Master Treatment Plan.

Intervention: Therapist will support the development of therapeutic rapport by creating a safe, nonjudgmental space for open communication, encouraging client engagement, and facilitating introductions to peers and staff to promote connection within the treatment community.

06/01/2026: Status - (Open)

06/09/2026: Status - (Attained) Goal met. Treatment plan created.

Objective: Client will begin to build therapeutic rapport with their therapist, staff, and peers, and demonstrate improved insight into addiction through psychoeducation over the course of the treatment plan period.

Intervention: Primary therapist will provide psychoeducation on addiction, recovery concepts, and program expectations through individual sessions and group facilitation to increase client's knowledge and insight into their substance use and treatment process.

06/01/2026: Status - (Open)

06/09/2026: Status - (Attained) Goal met. Client is active in individual and group sessions.

Treatment Plan Addendum

Level of Care:

- ☒ PHP (2.5)
- ☐ IOP 19 (2.1)
- ☐ IOP 5 (2.1)
- ☐ IOP 3 (2.1)
- ☐ OP (1.0)

Treatment Interventions to Support Established Treatment Plan:

- ☐ Individual Sessions
- ☐ 1x per week
 - ☒ 2x per week
 - ☐ Other:
- ☐ Group Sessions
- ☒ 5x per week
 - ☐ 4x per week
 - ☐ 3x per week
 - ☐ 2x per week
 - ☐ 1x per week
 - ☐ Other:

Client

May 01, 2026 01:59 PM

Staff

Caleb Madison, BS

May 01, 2026 01:59 PM

Fetching Report ...

Reviewer

Marleigh Johnson, MS, LPC
May 01, 2026 01:59 PM

Treatment Plan: 05/19/2026 -
05/19/2027

Primary Clinician: Marleigh Johnson

Expected Length of Treatment
(days): 365

Problem: Substance Use Disorders

Diagnostic Impression:

05/12/2026 - [F14.20] Cocaine use disorder, Severe

05/12/2026 - [F10.20] Alcohol use disorder, Severe

Behavioral Definitions:

05/12/2026 - [REDACTED] displays a pattern of maladaptive coping behaviors characterized by substance use that interferes with daily functioning.

Client Goal: [REDACTED] will develop the capacity to recognize, tolerate, and regulate intense emotions by learning and applying adaptive coping strategies in both group and individual therapy. She will work toward responding to emotional triggers with intentional, skill-based behaviors rather than impulsive reactions, leading to increased stability in daily functioning and reduced emotional overwhelm.

Objective: [REDACTED] will identify emotional triggers and physiological signs of distress during group sessions.

Intervention: The clinician will use CBT-based emotional awareness exercises in individual sessions to help [REDACTED] label emotions, identify triggers, and track patterns.

06/20/2026: Status - (Open)

Objective: [REDACTED] will practice mindfulness and grounding techniques in individual therapy to manage emotional intensity.

Intervention: The clinician will facilitate mindfulness and grounding practice in group therapy, including guided breathing, body scans, and distress-tolerance exercises.

06/20/2026: Status - (Open)

Client Goal: [REDACTED] will build healthier, more stable interpersonal relationships by strengthening her communication skills, increasing emotional awareness in relational contexts, and reducing patterns of conflict, withdrawal, or dependency. Through active participation in group and individual sessions, she will practice assertive communication and develop insight into relational patterns that undermine connection and trust.

Objective: [REDACTED] will learn and apply assertive communication skills in group sessions.

Intervention: The clinician will lead communication-skills training in group therapy, including structured role-plays and feedback to reinforce assertiveness and active listening.

06/19/2026: Status - (Open)

Objective: [REDACTED] will explore personal relationship patterns and emotional needs in group and individual sessions.

Intervention: The clinician will use Interpersonal Therapy (IPT) techniques to help [REDACTED] examine relational patterns, attachment themes, and emotional needs.

06/19/2026: Status - (Open)

Client Goal: [REDACTED] will decrease impulsive behaviors and reduce reliance on substances as a coping mechanism by identifying triggers, increasing distress tolerance, and implementing alternative coping strategies learned in treatment. Through consistent engagement in therapy, she will work toward maintaining sobriety, improving decision-making, and restoring stability in her personal, occupational, and relational life.

Objective: [REDACTED] will identify triggers for impulsive decisions and substance use during group and individual therapy.

Intervention: The clinician will use motivational interviewing to explore triggers, increase insight, and strengthen [REDACTED]'s commitment to behavioral change.

06/19/2026: Status - (Open)

Objective: [REDACTED] will develop coping strategies for managing stress and emotional pain in group and individual sessions.

Intervention: The clinician will teach DBT distress-tolerance skills in group therapy and guide [REDACTED] through real-time practice and feedback.

06/19/2026: Status - (Open)

Treatment Plan Addendum

Level of Care:

- ☒ PHP (2.5)
- ☐ IOP 19 (2.1)
- ☐ IOP 5 (2.1)
- ☐ IOP 3 (2.1)
- ☐ OP (1.0)

Treatment Interventions to Support Established Treatment Plan:

- ☒ Individual Sessions
 - ☐ 1x per week
 - ☒ 2x per week
 - ☐ Other:
- ☒ Group Sessions
 - ☒ 5x per week
 - ☐ 4x per week
 - ☐ 3x per week
 - ☐ 2x per week
 - ☐ 1x per week
 - ☐ Other:

Client

...

May 19, 2026 07:47 AM

Staff

Marleigh Johnson, MS, LPC
May 19, 2026 07:47 AM

Treatment Plan: 05/19/2026 -
05/19/2027

Primary Clinician: Marleigh Johnson

Expected Length of Treatment
(days): 365

Problem: Substance Use Disorders

Diagnostic Impression:

05/12/2026 - [F14.20] Cocaine use disorder, Severe

05/12/2026 - [F10.20] Alcohol use disorder, Severe

Behavioral Definitions:

05/12/2026 - [REDACTED] displays a pattern of maladaptive coping behaviors characterized by substance use that interferes with daily functioning.

Client Goal: [REDACTED] will develop the capacity to recognize, tolerate, and regulate intense emotions by learning and applying adaptive coping strategies in both group and individual therapy. She will work toward responding to emotional triggers with intentional, skill-based behaviors rather than impulsive reactions, leading to increased stability in daily functioning and reduced emotional overwhelm.

Objective: [REDACTED] will identify emotional triggers and physiological signs of distress during group sessions.

Intervention: The clinician will use CBT-based emotional awareness exercises in individual sessions to help [REDACTED] label emotions, identify triggers, and track patterns.

Intervention: Mindfulness-based activities (e.g., guided breathing, body scans) practiced in both group and individual settings to enhance emotional awareness.

Intervention: Mindfulness-based activities (e.g., guided breathing, body scans) practiced in both group and individual settings to enhance emotional awareness.

06/20/2026: Status - (Open)

08/08/2026: Status - (Extended) Client is able to identify some primary emotional triggers, especially related to interpersonal conflict and relationship dynamics. She continues to work on recognizing early warning signs of distress consistently and requires ongoing support to improve awareness prior to escalation.

Objective: [REDACTED] will practice mindfulness and grounding techniques in individual therapy to manage emotional intensity.

Intervention: Interpersonal Therapy (IPT) to [REDACTED]

Intervention: Communication skills training i [REDACTED]

Intervention: The clinician will facilitate mindfulness and grounding practice in group therapy, including guided breathing, body scans, and distress-tolerance exercises.

06/20/2026: Status - (Open)

08/08/2026: Status - (Extended) Client has been introduced to and actively participates in mindfulness and grounding interventions in session. She has not reported using these skills outside of sessions.

Client Goal: [REDACTED] will build healthier, more stable interpersonal relationships by strengthening her communication skills, increasing emotional awareness in relational contexts, and reducing patterns of conflict, withdrawal, or dependency. Through active participation in group and individual sessions, she will practice assertive communication and develop insight into relational patterns that undermine connection and trust.

Objective: [REDACTED] will learn and apply assertive communication skills in group sessions.

Intervention: The clinician will lead communication-skills training in group therapy, including structured role-plays and feedback to reinforce assertiveness and active listening.

06/19/2026: Status - (Open)

08/08/2026: Status - (Extended) Client continues to work on implementing these skills in real-life situations, particularly within interpersonal and romantic relationships.

Objective: [REDACTED] will explore personal relationship patterns and emotional needs in group and individual sessions.

Intervention: The clinician will use Interpersonal Therapy (IPT) techniques to help [REDACTED] examine relational patterns, attachment themes, and emotional needs.

06/19/2026: Status - (Open)

08/08/2026: Status - (Extended) Client continues to explore maladaptive relationship patterns and demonstrates growing insight into dependency, boundary issues, and emotional needs. Despite this progress, she continues to struggle with behavior change and remains vulnerable to relational stressors.

Client Goal: [REDACTED] will decrease impulsive behaviors and reduce reliance on substances as a coping mechanism by identifying triggers, increasing distress tolerance, and implementing alternative coping strategies learned in treatment. Through consistent engagement in therapy, she will work toward maintaining sobriety, improving decision-making, and restoring stability in her personal, occupational, and relational life.

Objective: [REDACTED] will identify triggers for impulsive decisions and substance use during group and individual therapy.

Intervention: The clinician will use motivational interviewing to explore triggers, increase insight, and strengthen [REDACTED]'s commitment to behavioral change.

06/19/2026: Status - (Open)

08/08/2026: Status - (Extended) Client continues to work on identify key triggers for impulsivity and substance use, including emotional dysregulation and high-risk environments. She remains vulnerable to acting on these triggers and continues to require structure and support to reduce risk.

Objective: [REDACTED] will develop coping strategies for managing stress and emotional pain in group and individual sessions.

Intervention: The clinician will teach DBT distress-tolerance skills in group therapy and guide [REDACTED] through real-time practice and feedback.

06/19/2026: Status - (Open)

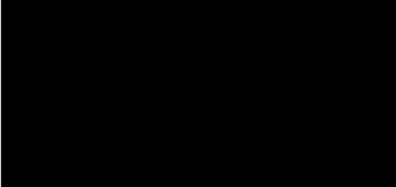
08/08/2026: Status - (Extended) Client has been introduced to a variety of coping strategies and is beginning to utilize them with support; however, she continues to experience difficulty effectively managing stress and emotional discomfort independently, indicating need for continued skill development.

Client



May 19, 2026 07:47 AM

Staff	
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Marleigh Johnson, MS, LPC
May 19, 2026 07:47 AM

----- END OF TX PLAN -----

TREATMENT PLAN REVIEW

-- No Treatment Plan Reviews --

----- END OF TX PLAN REVIEW -----

Fetching Report ...

DISCHARGE / TRANSFER

ClientName:

[REDACTED]

Age:

24 Years

Date of Birth:

12/08/2001

Admit Date:

05/01/2026

Length Of Stay:

47 days

Discharge Date:

May 1 2026 12:00AM

Master Discharge/Transfer Overview

Version	Author	Created Date
MDP_MDP_May 1 2026 1:47PM (1)	Caleb Madison, BS	05/01/2026 1:47PM

Reason for Admission:

Explanation For Changes in Diagnosis, if any:

Strengths:

I care a lot about people. I am very empathetic. I give good advice.. I am very smart and kind.

Needs:

I want to use those strengths on myself. Setting boundries with myself. I give all of my self to others and I get lost.

Abilities:

I can sing, dance, play guitar and piano.

Goals (Preferences):

I want to be okay with who I am. I don't want to always be chasing the next best thing.

Anticipated Discharge Date:

05/01/2026

Reason for Discharge:

Currently In Treatment

Discharge Transition Obstacles:

Family/Guardian Participation in Treatment:

Critical Events & Interaction:

Prognosis:

Living Arrangements:

Services Provided:

Recommendations:

Summary of Treatment Goals:

Active Meds:

Drug	Instruction
Apri 0.15 mg-0.03 mg tablet	Take 0.03-0.15 mg by mouth
bupirone 10 mg tablet	2x per day 20 mg total per day Take 10 mg by mouth
levothyroxine 75 mcg tablet	Take 75 mcg by mouth
Lexapro 20 mg tablet	Take 20 mg by mouth
semaglutide 0.25 mg or 0.5 mg (2 mg/3 mL) subcutaneous pen injector	Take 0.1 Unit(s) subcutaneous
trazodone (bulk) powder	Take 50 Capsule(s) miscellaneous 2x per day

Diagnosis:

Diagnostic Impression	Priority	Created By	Created Date	Resolved
[F14.20] Cocaine use disorder, Severe	Primary	Caleb Madison	05/01/2026	--
[F10.20] Alcohol use disorder, Severe	Secondary	Caleb Madison	05/01/2026	--

Fetching Report ...

Staff

Caleb Madison, BS
May 1 2026 2:47PM -04:00

----- END OF DISCHARGE / TRANSFER -----

DISCHARGE SUMMARY

ClientName: [REDACTED]	Age: 24 Years	Date of Birth: 12/08/2001
Admit Date: May 1 2026	Length Of Stay: 47 days	Discharge Date: N/A

----- END OF DISCHARGE SUMMARY -----

OTHER NOTES

-- No Notes --